

ESOPHAGEAL DISORDERS — Clinical Revision Table

Bold = high-yield. Dr Fahmi Kakamad. Covers GERD, motility disorders, perforation, carcinoma.

GERD

Overview	Prevalence 8–20%; abnormal LES opening → mucosal damage . Assoc: hiatal hernia, scleroderma, asthma, COPD, DM, HRT, coeliac
Complications	Esophagitis, erosions, strictures , Barrett's esophagus , adenocarcinoma
Alarm symptoms	Dysphagia , odynophagia , GI bleed , weight loss , iron-def anaemia , early satiety , vomiting , choking → always endoscope
Diagnosis	Clinical (heartburn) → empiric PPI trial (response = dx) . Endoscopy + biopsy for complications/Barrett's. pH monitoring if refractory. Barium not routine
Treatment	Self-care (antacid/H2/PPI) + lifestyle → 2wk no response → empiric PPI → alarm/refractory → endoscopy. Lifestyle: weight loss, elevate bed 6–8", small meals, no eating 3h pre-sleep, stop alcohol/tobacco. Anti-reflux surgery = last resort

MOTILITY DISORDERS

Disorder	Pathophysiology	Symptoms	Diagnosis	Treatment
Achalasia	Failure of LES to relax + dilated lower 2/3 (denervation; T. cruzi = Chagas)	Progressive dysphagia (solids+liquids), regurgitation, nocturnal cough, weight loss	Barium = bird's beak; manometry = LES fails to relax ; endoscopy excludes other	CCB/nitrate (short); balloon dilatation (60–90%, perforation 1–5%) ; botox (90% but 50% recurrence); myotomy/POEM
Diffuse esophageal spasm	Vagal degeneration; female > male. Simultaneous, UN coordinated high-pressure contractions	Severe intermittent chest pain, dysphagia, food impaction	Barium = corkscrew/pseudodiverticulum ; manometry simultaneous multipeaked (120 mmHg / >2.5s)	Acid suppression, nitrate, CCB, sedative, botox. Surgery (long esophagomyotomy) if severe
Nutcracker (hypercontractile)	Excessively strong but COORDINATED peristalsis (unlike DES)	Severe squeezing chest pain (mimics angina), dysphagia solids+liquids	Manometry = high amplitude >180 mmHg + normal LES relaxation ; endoscopy normal	Avoid triggers; CCB (nifedipine), nitrate, peppermint oil, low-dose antidepressant ; refractory → botox/POEM

PERFORATION & CARCINOMA

	Cause / risk	Features / diagnosis / treatment
Esophageal perforation	Traumatic (common): stent placement = most common (25%), endoscopy/dilation, vomiting (Boerhaave), trauma . Non-traumatic: neoplasm, caustic. → mediastinitis, pneumomediastinum, empyema, sepsis	Lower chest/upper abd pain (→L shoulder/back), vomiting/haematemesis, subcutaneous emphysema , fever, tachycardia, shock. CXR: hydrothorax L>R, pneumomediastinum, mediastinal widening . Gastrografin first (no contrast if sedated). Mgmt: ICU, NPO, NG suction, broad-spectrum Abx, early surgery <24h
Esophageal carcinoma	SCC = upper/mid, tobacco + alcohol. Adenocarcinoma = lower, obesity/GERD/Barrett's	Dx: endoscopy + biopsy . CT/PET, EUS (depth/staging) . Tx: early = cure (esophagectomy or endoscopic resection EMR/ESD); late = palliative; chemo-radiotherapy pre/post